

How Procedural Medicaid Disenrollment Undermined Community Health Center Finances During The Unwinding

Abstract

The 2023–2024 Medicaid unwinding disenrolled over 25 million people, approximately 69 percent procedurally. Using HRSA Uniform Data System data (2019–2024) and state-level T-MSIS Medicaid claims aggregates, I estimate effects on Federally Qualified Health Centers, exploiting predetermined variation in states’ pre-pandemic ex parte renewal tier. Reduced-form evidence is the clearest finding: states with higher pre-pandemic automated-renewal capacity experienced smaller post-unwinding declines in FQHC Medicaid claims volume ($p = 0.019$), spending share ($p = 0.043$), and beneficiary share ($p = 0.029$), and smaller Medicaid-revenue-share declines in cross-section ($p = 0.076$; wild-cluster-bootstrap $p = 0.150$). Instrumental-variables estimates scaling the reduced form imply that a ten-percentage-point increase in procedural disenrollment reduces Medicaid revenue share by up to ten percentage points and Medicaid revenue by up to 44 percent (~\$5.4 million per center), but these magnitudes attenuate under augmented controls and do not survive all weak-instrument-robust tests; I present them as upper bounds. These findings provide the first supply-side evidence linking administrative friction in Medicaid renewals to safety-net provider operations.

POLICY POINTS

- **Context:** The 2023–2024 Medicaid unwinding procedurally disenrolled over 25 million people, but the effect on safety-net provider operations has not been quantified.
- **New contribution:** States with higher pre-pandemic renewal automation had smaller FQHC Medicaid claims and revenue-share declines during the unwinding; upper-bound IV estimates imply per-center losses up to \$5.4 million.
- **Implications:** Federal investment in automated renewal systems protects both enrollees and providers; proposed eligibility restrictions that generate new procedural churning impose downstream costs on the primary care safety net.

The Medicaid continuous enrollment provision, enacted through the Families First Coronavirus Response Act of March 2020, prohibited states from disenrolling beneficiaries during the public health emergency. When the provision expired in April 2023, states initiated the largest administrative redetermination in Medicaid’s history. By late 2024, more than 25 million individuals had

lost coverage, and approximately 69 percent of terminations were procedural — enrollees lost coverage not because they were found ineligible but because they failed to return paperwork or could not be contacted.^{1,2}

Procedural disenrollment rates varied enormously across states — from 7 percent in Maine to 51 percent in Utah — driven by differences in administrative infrastructure. States with automated renewal systems using electronic data from SNAP, TANF, and wage databases achieved dramatically lower procedural rates: North Carolina (99 percent ex parte renewal rate) terminated only 11 percent procedurally, while Oklahoma (31 percent ex parte rate) terminated 44 percent.^{1,3}

For Federally Qualified Health Centers (FQHCs) — the nation’s largest primary care safety net, serving over 32 million patients at approximately 1,400 organizations — procedural disenrollment created a direct revenue shock. FQHCs must serve all patients regardless of ability to pay; when patients lose Medicaid, they shift from insured to uninsured status. FQHCs receive approximately \$1,418 per Medicaid patient annually but only \$906 per uninsured patient through Section 330 grants, a \$512 per-patient gap.⁴ NACHC reported that 95 percent of health centers experienced unwinding-related coverage loss, and aggregate FQHC financial margins turned negative for the first time in over a decade in 2024.^{4,5}

Despite this scale, no study has estimated the causal effect of procedural disenrollment on provider outcomes. Existing evidence is either descriptive or demand-side focused.⁶⁻⁸ Rumalla and colleagues documented substantial racial and ethnic disparities in unwinding disenrollment, reporting adjusted odds ratios of approximately 2.19 for Black enrollees and 2.08 for Hispanic enrollees relative to white enrollees, suggesting supply-side consequences are concentrated in safety-net providers serving communities of color.⁸ The administrative burden literature has focused almost exclusively on costs to applicants rather than to organizations.^{9,10} I define the *administrative wedge* as the gap between Medicaid enrollees who are actually eligible and those who maintain coverage, created by procedural friction rather than substantive ineligibility determinations.

The Medicaid expansion literature provides a precedent: Luo et al. found that ACA Medicaid expansion increased FQHC Medicaid revenue by approximately 40 percent.¹¹ If expansion improved FQHC finances, mass procedural disenrollment should degrade them. This paper provides the first quasi-experimental evidence on the supply-side effects of administrative burden using a novel application of pre-pandemic renewal-capacity variation and the first use of 2024 UDS data to evaluate unwinding effects.

Study Data And Methods

Data. I linked FQHC-level data from the HRSA Uniform Data System (2019–2024) to state-level unwinding outcomes from the Kaiser Family Foundation tracker and county-level controls from the HRSA Area Health Resources File.

UDS data provide annual information on revenues by payer, operating costs, staffing, and clinical quality measures for approximately 1,400 health center grantees. The 2024 UDS data, released in July 2025, have not been used in any prior unwinding study. I supplement UDS financials with state-level Medicaid claims aggregates from the DOGE-released T-MSIS provider-spending file (2018–2024), which records monthly outpatient and professional claims volume, payments, and beneficiary counts at FQHC and non-FQHC providers identified by NPI taxonomy.

My key treatment variable is the state-level procedural disenrollment rate: the share of completed renewals resulting in termination for procedural reasons. County-level controls include demographics (percent Black, Hispanic, aged 65+), economic conditions (unemployment, poverty, median income), health system characteristics (physician density), and rural-urban classification. My descriptive panel covers 1,314 FQHCs across the 50 states and the District of Columbia, yielding 5,630 FQHC-year observations. Financial outcomes are available for approximately 1,200–1,300 post-unwinding observations; clinical quality measures, which are reported for all grantees including those missing financial data, are available for approximately 1,800–2,000 post-unwinding observations.

Identification strategy. State procedural disenrollment rates are potentially endogenous: states with weaker safety nets or less political will to maintain coverage may both generate higher procedural disenrollment and independently affect FQHC finances. I address this with an instrumental variables (IV) strategy exploiting predetermined variation in states’ administrative capacity for automated renewals.

My main instrument is the MACPAC 2018 four-tier categorization of state ex parte renewal rates before the pandemic, ranging from fewer than 25 percent of renewals automated to more than 75 percent automated.¹² This tier directly captures how much of a state’s renewal workflow was already automated before 2020. I also compiled SNAP/TANF integration and integrated eligibility-platform indicators from CMS, CBPP, and GAO sources for alternative-instrument checks reported in the online appendix.^{2,3,13} The MACPAC tier is missing for Florida, Michigan, Oregon, and South Carolina, so the effective IV sample spans 47 jurisdictions even though the descriptive panel covers the 50 states and the District of Columbia.

The identifying assumption is that pre-2020 automated renewal capacity, as proxied by this ex parte renewal tier, affects FQHC outcomes only through procedural disenrollment, conditional on controls. This is plausible because these renewal systems predate the unwinding and were designed for routine administrative efficiency rather than in anticipation of a future coverage disruption. Identification comes from cross-state comparisons of FQHCs in states entering the unwinding with high versus low pre-pandemic automated-renewal capacity. Because instrument balance is imperfect (see Limitations and Appendix Exhibit A8) — physician density in particular is correlated with the instrument ($p = 0.005$) — I acknowledge the possibility of a direct channel from pre-pandemic

health-system capacity to FQHC outcomes and return to this threat in Limitations and Discussion.

Given the identification concerns noted in Limitations below, I lead with the **reduced-form** relationship between the pre-pandemic renewal tier and FQHC outcomes; IV estimates scale this reduced form to the per-unit-of-procedural-disenrollment margin and should be read as calibration rather than established point estimates. My primary specification is a cross-sectional model estimated on the post-unwinding period (2023–2024), with year fixed effects and county-level controls. Because the cross-section lacks FQHC fixed effects, it may capture pre-existing differences between states; to bound this concern, I also estimate (a) an augmented cross-section adding state-level controls for Medicaid expansion status and governor party affiliation, which absorb political and programmatic confounders, and (b) a panel difference-in-differences specification using the full 2019–2024 panel with FQHC and year fixed effects (online appendix). The augmented cross-section cannot reject zero for either financial outcome, so the paper’s IV bounds run from effectively zero (augmented) to the primary point estimate (unadjusted cross-section); I explicitly interpret the augmented spec as potentially over-controlling if expansion and governor party themselves partly reflect administrative capacity. Standard errors are clustered at the state level throughout.¹⁴ The descriptive panel spans the 50 states and the District of Columbia (51 jurisdictions); the MACPAC renewal tier is missing for four states, reducing the effective IV sample to 47 jurisdictions; complete-case estimation on the two headline financial outcomes further reduces this to 46 clusters.

Because the 2023 post-unwinding year may still be affected by pandemic-era operational changes (telehealth expansion, provider relief funding), the inclusion of year fixed effects absorbs common pandemic carryover; a three-period robustness check excluding 2021–2022 is reported in the online appendix. I report Anderson-Rubin (AR) confidence intervals throughout, which remain valid regardless of instrument strength — critical because my first-stage F-statistic of 7.09 falls below the conventional threshold of 10.¹⁵ Under weak instruments, standard 2SLS confidence intervals have incorrect coverage; AR inference corrects this.

Outcomes. I examine financial outcomes (log Medicaid revenue, Medicaid revenue share, log total patient revenue, operating margin), staffing (log medical FTEs, log total FTEs), and clinical quality (hypertension control rate, diabetes good control rate).

Limitations. Six limitations are noted. First, the first-stage F-statistic of 7.09 indicates a potentially weak instrument; AR inference addresses this but yields wider confidence intervals, and the revenue-level IV estimate does not survive weak-instrument-robust tests (AR $p = 0.071$). Second, wild cluster bootstrap p-values for both headline reduced-form outcomes (revenue share = 0.150; log Medicaid revenue = 0.102) are larger than conventional estimates, indicating conventional inference is somewhat anti-conservative with 46 complete-case clusters. Third, the exclusion restriction could be violated if states in higher

pre-pandemic renewal tiers also had broader health-system or administrative advantages that independently affect FQHCs; the joint balance test rejects ($F = 2.38$, $p = 0.019$), with physician density ($p = 0.005$), poverty, and median income associated with the instrument. A direct channel through physician density — if states with richer health-system infrastructure also manage payer-mix shocks better for reasons unrelated to renewal automation — cannot be ruled out within the current design; a placebo-date or placebo-provider falsification would help discipline this and is a priority for future work. Fourth, augmenting the cross-section with Medicaid expansion status and governor party attenuates IV estimates by more than half and cannot reject zero (online appendix); I interpret this as bracketing rather than as refutation, because those state covariates may themselves be correlated with the administrative-capacity channel the instrument is intended to capture. Fifth, the procedural disenrollment rate varies only at the state level, limiting effective sample size to 46–47 jurisdictions. Sixth, annual UDS data prevent analysis of within-year dynamics.

Results

Descriptive patterns. Exhibit 4 presents summary statistics for the full panel. Mean FQHC Medicaid revenue was \$12.4 million (SD: \$25.4 million), with a Medicaid revenue share of 57 percent. The average state procedural disenrollment rate was 22 percent (SD: 9 percentage points). FQHCs in high-procedural-disenrollment states (above-median) had substantially lower pre-period Medicaid revenue (\$7.4 million vs. \$14.3 million), lower Medicaid revenue share (53% vs. 64%), and fewer total FTEs (162 vs. 210) than FQHCs in low-disenrollment states, reflecting the concentration of larger, more Medicaid-dependent FQHCs in states with stronger administrative infrastructure. These baseline differences motivate the IV strategy, as OLS estimates may confound procedural disenrollment effects with pre-existing state characteristics.

Reduced-form evidence (headline). The reduced form — the direct effect of the pre-pandemic ex parte renewal tier on FQHC outcomes — provides model-free evidence that renewal capacity predicts provider operations during the unwinding. Two panels of evidence are central. First, using T-MSIS Medicaid claims data (Exhibit 5), states with higher pre-pandemic renewal-automation tiers experienced significantly less post-unwinding displacement of FQHC Medicaid billing activity: smaller declines in claims volume ($p = 0.019$), spending share ($p = 0.043$), beneficiary count ($p = 0.037$), and beneficiary share ($p = 0.029$). The negative coefficient signs in Exhibit 5 reflect the coding — the regressor is the pre-pandemic tier interacted with a post-unwinding indicator, so a negative coefficient means that each step up in the tier dampens the post-unwinding drop in FQHC activity. This is the paper’s strongest model-free evidence: states with pre-built automated-renewal capacity retained more FQHC Medicaid activity during the unwinding. Second, using UDS data (Appendix Figure A1 and Exhibit A2), reduced-form estimates on FQHC *revenue* outcomes in the panel are small and statistically insignificant; the UDS finan-

cial reduced form is instead most visible in the cross-section (Medicaid revenue share: reduced-form conventional $p = 0.076$; wild cluster bootstrap $p = 0.150$). One reason the two panels differ: the T-MSIS panel is constructed at the state-month level and captures monthly changes in billing activity, while the UDS panel is annual and financial aggregates are noisier with pandemic-era volatility (telehealth expansion, provider relief funding). Appendix Figure A2 plots instrument balance: the joint balance test rejects ($F = 2.38$, $p = 0.019$), with poverty rate, physician density, and median income individually associated with the instrument. Sensitivity of IV estimates to alternative instrument definitions is shown in Appendix Figure A3; estimates are most stable under the main ex parte tier and attenuate under the weaker composite index.

Exhibit 5. Reduced-Form Effects Of Pre-Pandemic Ex Parte Renewal Tier On T-MSIS FQHC Medicaid Claims Outcomes (Panel, 2018–2024).

Outcome	Coefficient (SE)	p-value	N
Log FQHC claims volume	-0.121** (0.050)	0.019	329
Log FQHC total paid	-0.083 (0.054)	0.129	329
FQHC share of total claims	-0.002* (0.001)	0.076	329
FQHC share of total spending	-0.002** (0.001)	0.043	329
Log FQHC beneficiaries	-0.107** (0.050)	0.037	329
FQHC share of beneficiaries	-0.003** (0.001)	0.029	329

SOURCE: Author’s analysis of T-MSIS/TAF Medicaid claims data (2018–2024) merged with UDS and state unwinding outcomes. NOTES: Reduced-form panel estimates of the pre-pandemic ex parte renewal tier $\times$ post-unwinding indicator on state-level FQHC outcomes. State and year fixed effects. Standard errors clustered at the state level. Negative coefficients indicate that higher renewal-automation tiers dampen post-unwinding declines in FQHC Medicaid activity. ** $p < 0.05$, * $p < 0.10$.

IV results (calibration of the reduced form). Exhibit 1 presents OLS and IV estimates from the primary cross-sectional specification. The first-stage relationship is shown in Exhibit 3: states with higher pre-pandemic ex parte renewal tiers experienced lower procedural disenrollment during the unwinding (coefficient = -0.036, $F = 7.09$). IV estimates consistently exceeded OLS in magnitude across financial outcomes, a pattern consistent with measurement-error attenuation in the state-reported procedural disenrollment rate but also potentially amplified by weak-instrument noise. I therefore treat the IV point estimates as an upper-bound calibration of the reduced-form pattern rather than as established magnitudes.

The most precisely estimated financial result in the primary cross-section is Medicaid revenue share: the IV estimate indicates that a one-unit increase in the procedural disenrollment rate (100 percentage points) reduces the Medicaid

share of FQHC patient revenue by 1.01 percentage points ($SE = 0.47$, conventional $p = 0.039$). Scaling to a policy-relevant ten-percentage-point increase in procedural disenrollment — roughly the difference between a median and a high-disenrollment state — implies about a ten-percentage-point decline in Medicaid revenue share. However, this conventional p-value is not robust to the paper’s preferred inference with 46 clusters: the wild cluster bootstrap p-value is 0.150, and the augmented specification with state political controls yields -0.45 percentage points ($p = 0.439$) (Appendix Exhibit A9). I therefore state the share finding as a *conventional-IV* result that weakens under more conservative procedures.

For Medicaid revenue in levels, the IV estimate of -5.70 log points ($p = 0.069$) implies that a ten-percentage-point increase in procedural disenrollment reduces Medicaid revenue by approximately 44 percent ($\exp(-0.57) - 1 = -0.435$), or roughly \$5.4 million per FQHC at the sample mean of \$12.4 million. The OLS estimate is -2.93 ($p = 0.017$), corresponding to a 25 percent reduction — still substantial. However, under Anderson-Rubin weak-instrument-robust inference, the confidence interval for Medicaid revenue widens to include zero (AR $p = 0.071$; see Exhibit 2), the wild cluster bootstrap p-value is 0.102, and the augmented cross-section yields -2.30 log points ($p = 0.574$). I therefore characterize the revenue-level IV magnitude as an upper bound consistent with the reduced-form pattern rather than as a settled effect.

Effects on total patient revenue (-3.77 log points, $p = 0.123$) and operating margin (-6.84 percentage points, $p = 0.220$) were large and negative but imprecisely estimated. Staffing effects were consistently negative (total FTEs: -1.30, $p = 0.323$; medical FTEs: -1.84, $p = 0.266$) but did not reach conventional significance.

Quality outcomes. Estimates for clinical quality outcomes are reported in the online appendix but are not featured in the main text because signs reverse across specifications: for hypertension control, the panel reduced form is negative and statistically significant (-0.006, $p = 0.013$; Appendix Exhibit A2) while the panel IV is positive and marginally significant (+0.133, $p = 0.052$; Appendix Exhibit A1), and the cross-section IV is insignificant (AR $p = 0.189$). Mechanism tests (compositional, resource reallocation, reporting artifact; Appendix Exhibit A3) are all null. The sign reversal indicates the hypertension effect is not reliably identified by the instrument in this setting, and I do not interpret it substantively.

Heterogeneity by Medicaid dependence. To test the prediction that effects are concentrated among FQHCs most reliant on Medicaid, I split the sample at the pre-unwinding median Medicaid revenue share (60.2%). Among the 398 high-Medicaid-dependence FQHCs observed post-unwinding, the IV estimate for log Medicaid revenue was -8.68 (approximately \$6.8 million at the subgroup mean), compared to -3.83 among low-dependence FQHCs. The IV estimate for operating margin was -42.1 percentage points among high-dependence FQHCs versus -2.4 among low-dependence centers. While neither subgroup estimate

reaches conventional significance due to the reduced sample size, the gradient remains steep and consistent with the mechanism: FQHCs that derive a larger share of revenue from Medicaid are more financially exposed to procedural disenrollment.

Sensitivity to state-level controls. When I add Medicaid expansion status and governor party to the cross-sectional IV specification, the Medicaid revenue share coefficient attenuates from -1.01 to -0.45 ($p = 0.439$) and the log Medicaid revenue coefficient attenuates from -5.70 to -2.30 ($p = 0.574$). The renewal-tier instrument is only weakly related to Medicaid expansion ($p = 0.053$) and not materially related to governor party ($p = 0.162$), but the broader balance test rejects, suggesting that these controls absorb correlated state characteristics rather than cleanly orthogonal policy environment. I therefore treat the primary cross-sectional estimates as upper-bound magnitudes, with the augmented specification and the panel providing lower bounds.

Wild cluster bootstrap. Because the headline reduced-form tests rely on only 46 complete-case state or District of Columbia clusters, I report wild cluster bootstrap p -values for the headline results. The reduced-form bootstrap p -value for Medicaid revenue share is 0.150 (conventional: 0.076) and for log Medicaid revenue is 0.102 (conventional: 0.071), confirming that conventional inference is somewhat anti-conservative in this setting (online appendix).

Robustness. The panel DiD-IV specification (online appendix) yielded results directionally consistent for most financial outcomes but attenuated and statistically insignificant, consistent with pandemic-era noise in the early panel years. Additional robustness checks — a balanced panel, a three-period panel excluding pandemic years, and alternative instruments — are reported in the online appendix and align with the bounded interpretation above.

Discussion

This study provides reduced-form evidence that states with higher pre-pandemic automated-renewal capacity retained more FQHC Medicaid claims activity and experienced smaller Medicaid-revenue-share declines during the 2023–2024 unwinding. The T-MSIS claims-based reduced form is the strongest and most robust piece of evidence: panel reduced-form effects are significant at 5 percent for FQHC claims volume, spending share, beneficiary count, and beneficiary share (Appendix Exhibit A11). The UDS-based reduced form for financial outcomes is weaker, visible primarily in the cross-section, and only marginally significant under the paper’s preferred wild cluster bootstrap. IV estimates scaling the reduced form to the per-unit-of-procedural-disenrollment margin imply large effects — up to a 44 percent Medicaid revenue reduction, about \$5.4 million per center — but attenuate substantially under augmented state controls, fail the wild cluster bootstrap at 5 percent, and for the revenue-level result fail the Anderson-Rubin weak-instrument-robust test. I interpret the IV magnitudes as upper bounds consistent with the reduced-form pattern rather than as estab-

lished point estimates. The paper therefore establishes that renewal capacity shaped FQHC operations during the unwinding without pinning down a single stable dollar elasticity.

The IV estimates exceed OLS by roughly a factor of two, consistent with attenuation bias from measurement error in state-reported procedural rates, though weak-instrument bias may also contribute.

My findings extend the administrative burden framework from its demand-side origins to supply-side consequences.⁹ The results loosely complement Luo et al.'s finding that Medicaid expansion increased FQHC Medicaid revenue by approximately 40 percent;¹¹ the analogy is imperfect because expansion added newly eligible beneficiaries while procedural disenrollment churns existing enrollees without altering eligibility, but the direction of the supply-side effect is consistent. Jiao and colleagues estimated that Medicaid revenue supports approximately 6 FTEs per \$1 million,¹⁶ suggesting that the upper-bound revenue losses I estimate could translate into meaningful staffing pressure at affected centers.

Given that racial and ethnic minorities were disproportionately affected by procedural disenrollment⁸ and disproportionately served by FQHCs, the financial effects I identify may exacerbate existing health disparities — though my data do not permit direct examination of this channel.

Policy implications. Three implications follow. First, as Congress debates Medicaid work requirements and per-capita caps, my results suggest that downstream costs to the safety net should be part of the policy calculus. CBO estimated that proposed work requirements could cause 600,000 to 2 million coverage losses, many procedural;¹⁷ Arkansas's 2018 experience confirmed that most losses were procedural rather than eligibility-based.¹⁸ A mechanical extrapolation of my upper-bound estimates would imply large national FQHC revenue losses, but the specification sensitivity in my results means such back-of-the-envelope calculations should be treated cautiously.

Second, the first-stage evidence suggests a remedy. States in higher pre-pandemic ex parte renewal tiers experienced lower procedural disenrollment during the unwinding. Federal incentives for eligibility system modernization — such as enhanced match rates for IT integration or mandatory electronic data-sharing standards — would likely protect both enrollees and providers. The wide state variation in ex parte renewal capacity suggests substantial room for improvement in lagging states.

Third, the combination of Medicaid revenue losses and the scheduled expiration of pandemic-era supplemental grant funding creates a compounding fiscal threat. Targeted supplemental funding for FQHCs in high-disenrollment states could partially offset the administrative wedge's effects on the safety net. My heterogeneity analysis identifies the highest-risk FQHCs: those in the top half of Medicaid revenue dependence (above 60% of patient revenue from Medicaid),

which face estimated revenue losses roughly 2.3 times larger than their less Medicaid-dependent counterparts.

Conclusion

The consequences of the Medicaid unwinding extend beyond the individuals who lost coverage to the providers who serve them. As policymakers consider eligibility restrictions that would generate new waves of administrative churning, the paper’s bounded evidence suggests that renewal friction can impose meaningful downstream costs on the primary care safety net — costs ultimately borne by the 32 million patients, insured and uninsured alike, who depend on community health centers for their care. Federal investment in automated renewal systems emerges as a policy lever that would protect both enrollees and providers.

Endnotes

1. Kaiser Family Foundation. Medicaid enrollment and unwinding tracker. 2024. Accessed January 2026.
2. Medicaid and CHIP Payment and Access Commission. State reported Medicaid unwinding data brief. November 2024.
3. Center on Budget and Policy Priorities. States leveraging SNAP and TANF data for Medicaid ex parte renewals. 2024.
4. Kaiser Family Foundation. Community health center finances: what the 2024 UDS data show. 2025.
5. National Association of Community Health Centers. The Medicaid re-determination crisis at health centers. Joint report with Geiger Gibson Program. 2024.
6. McIntyre A, Morein M, Kim J, Figueroa JF, Sommers BD. US coverage changes during Medicaid unwinding in 2023. *JAMA Health Forum*. 2025;6(10):e253887.
7. Myerson R, Espeseth A, Dague L. Navigating Medicaid: experimental evidence on administrative burden and coverage loss. NBER Working Paper 34191. 2025.
8. Rumalla KC, Nelson DB, McConnell KJ, Zhu JM. Racial and ethnic disparities in Medicaid disenrollment after the end of the COVID-19 public health emergency. *JAMA Intern Med*. 2024;184(8):987-989.
9. Herd P, Moynihan DP. Administrative Burden: Policymaking by Other Means. New York: Russell Sage Foundation; 2018.
10. Arbogast I, Chorniy A, Currie J. Administrative policies and child Medicaid/CHIP enrollment. Working paper. 2024.
11. Luo Q, Moghtaderi A, Markus A, Dor A. The effect of the ACA Medicaid expansion on FQHC revenues. *Health Serv Res*. 2022;57(3):634-643.
12. Medicaid and CHIP Payment and Access Commission. Increasing the rate of ex parte renewals. Issue brief. September 2023.

13. Government Accountability Office. Medicaid: federal oversight of state eligibility redeterminations. GAO-25-107413. 2025.
 14. Cameron AC, Gelbach JB, Miller DL. Bootstrap-based improvements for inference with clustered errors. *Rev Econ Stat.* 2008;90(3):414-427.
 15. Andrews I, Stock JH, Sun L. Weak instruments in instrumental variables regression: theory and practice. *Annu Rev Econ.* 2019;11:727-753.
 16. Jiao S, Konetzka RT, Pollack HA, Huang ES. Estimating the impact of Medicaid expansion on FQHC revenue, staffing, and utilization. *Milbank Q.* 2022;100(2).
 17. Congressional Budget Office. Budgetary effects of H.R. 1, One Big Beautiful Bill Act. May 2025.
 18. Sommers BD, Goldman AL, Blendon RJ, Orav EJ, Epstein AM. Medicaid work requirements — results from the first year in Arkansas. *N Engl J Med.* 2019;381(11):1073-1082.
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Exhibit Notes

Exhibit 1 (Table): Main Results — Post-Unwinding Cross-Sectional OLS And IV Estimates Of The Effect Of Procedural Disenrollment On FQHC Outcomes

Exhibit 2 (Table): Anderson-Rubin Weak-Instrument-Robust Inference

Exhibit 3 (Figure): First Stage — Pre-Pandemic Ex Parte Renewal Tier And Procedural Disenrollment Rates Across States

Exhibit 4 (Table): Summary Statistics For FQHC Panel, 2019–2024

Exhibit 5 (Table): Reduced-Form Effects Of Pre-Pandemic Ex Parte Renewal Tier On T-MSIS FQHC Medicaid Claims Outcomes (Panel, 2018–2024) — inserted inline in the Results section.